

TEXT OF LECTURE

“POLICY REFORMS AND REGULATORY CHALLENGES IN NIGERIA’S PHARMACEUTICAL SECTOR”

Presented by

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As Keynote Speaker

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Organized by the Pharmaceutical Association of Nigerian Students (PANS),
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PROTOCOL

It gives me great pleasure to be invited by the Comrade Abubakar Umar Abdullahi-led Executive Committee of the Pharmaceutical Association of Nigerian Students (PANS), Sa’adu Zungur Chapter, Sa’adu Zungur University, Gadau, Bauchi State, to deliver the keynote address on the occasion of your Pharmacy Week (Academic Pharmacy Summit).

Let me begin by congratulating the entire membership of PANS, Sa’adu Zungur Chapter, on this important occasion.

The Faculty of Pharmaceutical Sciences at Sa’adu Zungur University, Gadau, is relatively young, yet I am pleased to note that its students have already organized themselves into a vibrant professional association.

The topic assigned to me — **“Policy Reforms and Regulatory Challenges in Nigeria’s Pharmaceutical Sector”** — is both appropriate and timely.

As pharmacy students, it is essential that you understand the policy directions, reforms, and challenges within the profession you are preparing to enter. Pharmacy, as a healthcare profession, involves the scientific knowledge and practical expertise necessary for the **manufacture, formulation, storage, dispensing, and distribution of pharmaceutical products**, among other vital responsibilities.

2.0 THE NEED FOR HEALTH PROFESSIONS

The human body is designed in such a way that it may not always remain in perfect condition throughout life — whether in infancy, childhood, adolescence, adulthood, or old age. Over thousands of years, mankind has developed **curative, preventive, and diagnostic interventions** to address illnesses and maintain well-being.

Over time, specialization in healthcare delivery evolved — from the days of the ancient apothecaries to the sophisticated system we have today.

In Nigeria, the first legal framework for regulating healthcare professionals was established in **1927**, through the creation of the **Board of Medical Examiners (BME)** under British colonial administration.

From that single regulatory body in 1927, Nigeria now has **thirteen (13)** health regulatory agencies — a reflection of the growth and dynamism of the nation's healthcare sector.

These are:

1. **Pharmacy Council of Nigeria (PCN)** – for pharmacy and pharmacists
2. **Medical and Dental Council of Nigeria (MDCN)** – for medicine, dentistry, physicians, and dentists
3. **Nursing and Midwifery Council of Nigeria (NMCN)** – for nursing and midwifery
4. **Medical Laboratory Science Council of Nigeria (MLSCN)** – for medical laboratory science
5. **Community Health Officers Registration Board of Nigeria (CHORBN)** – for community health officers
6. **Radiographers Registration Board of Nigeria (RRBN)** – for radiography and radiographers
7. **Optometrists and Dispensing Opticians Registration Board of Nigeria (ODORBN)** – for optometry and opticians

8. **Medical Rehabilitation Therapists (Registration) Board of Nigeria (MRTB)** – for physiotherapy and rehabilitation therapists
9. **Health Records Officers Registration Board of Nigeria (HRORBN)** – for health records officers
10. **Dental Therapists Registration Board of Nigeria (DTRBN)** – for dental therapy
11. **Dental Technologists Registration Board of Nigeria (DTRBN)** – for dental technology
12. **Institute of Public Analysts of Nigeria (IPAN)** – for public analysts
13. **Institute of Chartered Chemists of Nigeria (ICCON)** – for chemistry and chemists

Each of these regulatory agencies operates under the supervision of the **Federal Ministry of Health (FMoH)** and was established by an Act of the National Assembly to oversee education, training, and professional practice in its respective field.

3.0 THE PHARMACY COUNCIL OF NIGERIA (PCN)

The **Pharmacy Council of Nigeria (PCN)** was established by the **Pharmacy Council of Nigeria (Establishment) Act, 2022**, to regulate and control pharmacy education, training, and practice in all aspects.

Predecessor bodies to the PCN include:

1. **Pharmacy Board of Nigeria (1936)**
2. **Pharmacists Board of Nigeria (PBN) (1964)**
3. **Pharmacists Council of Nigeria (PCN) (1992)**

3.1 REGULATION OF PHARMACY EDUCATION AND TRAINING

The PCN regulates pharmacy education and training through the **Education and Training Department** of its Registry and Head Office. The Council's responsibilities include:

- Accreditation of **PharmD** and **B.Pharm** programs in Nigerian universities

- Accreditation of **Pharmacy Technician programs** in colleges or schools of health technology
- Accreditation of **internship centres** in healthcare and pharmaceutical manufacturing facilities
- Supervision of the **Foreign Pharmacy Graduates Orientation Programme (FPGOP)**
- Coordination of **Mandatory Continuing Professional Development (MCPD)** programs in collaboration with universities and the National Institute for Pharmaceutical Research and Development (NIPRD), Abuja

All examination results arising from these programs are subject to review and approval by the Council before release.

3.2 REGULATION OF PHARMACY PRACTICE

Pharmacy practice is regulated by the Governing Council of the PCN through three core departments:

- **Inspection and Monitoring (I&M)**
- **Pharmacy Practice (PP)**
- **Enforcement**

Key reference materials include:

- *Compendium of Minimum Standards for the Provision of Pharmaceutical Care in Nigeria*
- *PCN Guidelines for the Issuance of Patent and Proprietary Medicine Vendor Licences (PPMVL)*
- *Manual for Pharmaceutical Inspectors*
- *Code of Ethics for Pharmacists in Nigeria*
- *PCN Sourcebook on Pharmacy Regulation in Nigeria*

Inspections and monitoring are conducted by:

- **State Pharmaceutical Inspectors Committees** chaired by Directors of Pharmaceutical Services

- **PPMVL Inspection Committees** in all states and the FCT
- **Hospital Pharmacy Inspection Committees**
- **Inspection teams for pharmaceutical manufacturing facilities**
- **PCN Enforcement Department** for compliance with licensing and operational standards

The overarching goal is to ensure that the public has access only to duly inspected, approved, registered, and licensed pharmaceutical premises. Those operating outside regulatory standards face appropriate sanctions.

4.0 CHALLENGES OF REGULATION AND POLICY REFORMS

In fulfilling its statutory mandate, the PCN encounters several challenges similar to those faced by other regulatory agencies. These include:

1. **Inadequate inspection personnel**, including registry staff
2. **Limited logistics**, especially vehicles for inspection, accreditation, and enforcement
3. **Insufficient funding** for staff training and development
4. **Jurisdictional overlaps** with other boards and agencies (e.g., NBTE, NVRC) due to unclear boundaries
5. **Periodic tensions** with professional associations related to pharmacy practice

Despite these challenges, the PCN continues to perform commendably. Over the past two decades, it has expanded its operational presence across the federation.

In 2003, there were only **four offices** — Abuja (Head Office and Registry), Enugu, Kano, and Lagos. Today, there are **forty-four (44)** state and zonal offices across Nigeria.

CONCLUSION

The Pharmacy Council of Nigeria has made remarkable progress in fulfilling its mandate to uphold professional standards and ensure quality healthcare delivery.

I commend the students of Sa'adu Zungur University for organizing this intellectually stimulating summit. Your choice of theme reflects foresight and a deep sense of professional responsibility.

Thank you for your kind attention and for the opportunity to share my thoughts with the students and faculty of this great institution — named after a great Nigerian.